PAM HEALTH AT HOME

Fax

To: MILLS, GAVIN	**From:** Pam Health at Home - Texas
Fax: 12109161359	**Pages:** 9
Phone: 210-916-7144	**Date:** 03-02-2026 5:20 PM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

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Opt-Out: Not Defined

Order No:485-26234

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
4C63A60DP86	12/30/2025	02/28/2026 - 04/28/2026	7580	747688

6. Patient's Name and Address
Perkins, John (7580)
10926 Gunsel Trail
San Antonio, TX 78245
Phone: (210) 854-9339

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 08/02/1956 **9. Sex:** ☑ M ☐ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Adult Low Dose Aspirin 81 mg tablet,delayed release [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Atorvastatin 10 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	2	daily	oral	LS
Cholecalciferol (vitamin D3) 25 mcg (1,000 unit) capsule ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 0 Capsule]	1	Daily	oral	LS
Cyclobenzaprine 10 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	3 times a day	oral	LS
Docusate sodium 100 mg capsule [Amount Of/ Dosage Form: 0 Capsule]	1	2 times a day	oral	LS
Doxazosin 1 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	at bedtime	oral	LS
Duloxetine 30 mg capsule,delayed release [Amount Of/ Dosage Form: 0 Capsule]	2	daily	oral	LS
Finasteride 5 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Flonase Allergy Relief 50 mcg/actuation nasal spray,suspension [Amount Of/ Dosage Form: 0 Spray]	1	2 times a day	nasal	LS
Hydrocodone 5 mg-acetaminophen 325 mg tablet [Amount Of/ Dosage Form: 0 Tablet]		every 4 hours	oral	LS - PRN/As Needed
Hydroxychloroquine 200 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1.5	daily	oral	LS
Iron (ferrous sulfate) 325 mg (65 mg iron) tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 0 Tablet]	1	Daily	oral	LS
Oxycodone 5 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	every 6 hours	oral	LS - PRN/As Needed
Peg 400-hypromellose-glycerin 1 %-0.2 %-0.2 % eye drops [Amount Of/ Dosage Form: 0 Metric Drop]	1 drop both eyes	2 times a day	ophthalmic (eye)	LS - PRN/As Needed
Propranolol 10 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	2 times a day	oral	LS

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Tylenol Extra Strength 500 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	every 4 hours	oral	LS - PRN/As Needed

11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
S42.201D	Unspecified fracture of upper end of right humerus, subsequent encounter for fracture with routine healing	12/30/2025	

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
M97.31XD	Periprosthetic fracture around internal prosthetic right shoulder joint, subsequent encounter		
S42.101D	Fracture of unspecified part of scapula, right shoulder, subsequent encounter for fracture with routine healing		
I10	Essential (primary) hypertension		
K70.30	Alcoholic cirrhosis of liver without ascites		
G47.33	Obstructive sleep apnea (adult) (pediatric)		
J90	Pleural effusion, not elsewhere classified		
G89.29	Other chronic pain		
M06.9	Rheumatoid arthritis, unspecified		
E66.9	Obesity, unspecified		
E78.5	Hyperlipidemia, unspecified		
W19.XXXD	Unspecified fall, subsequent encounter		
Z96.611	Presence of right artificial shoulder joint		
Z68.35	Body mass index [BMI] 35.0-35.9, adult		
Z91.81	History of falling		
Z79.82	Long term (current) use of aspirin		
Z79.891	Long term (current) use of opiate analgesic		

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14. DME and Supplies:
DME: Walker
ALCOHOL WIPES, NS GLOVES, PROBE COVERS, PPE MASK

15. Safety Measures:
911 Emergency / Protocol, Emergency Plan / Measures, Fall / Safety Precautions, Fall Precautions, Safety in ADLs, Standard Precautions/Infection Control, Transfer Precautions, Universal Precautions, Use of Assistive Devices, USE SLING ON RIGHT ARM PRN FOR COMFORT, Walker / Cane

16. Nutritional Req.
No Added Salt,
HEART HEALTHY

17. Allergies:
Medication Allergies:
ACE Inhibitors, amoxicillin, chocolate flavor, grape, Penicillins, Septra, Sulfa (Sulfonamide Antibiotics), trimethoprim

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation
☐ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☑ A- Dyspnea with minimal exertion
☑ B- Other (Specify): NWB to RUA

18.B. Activities Permitted
☐ 1- Complete Bedrest ☐ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☐ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☑ D- Other (Specify):SBA

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☑ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:
Frequency
SN Frequency: 1W3, 1EOW6 Starting Week of 03/01/2026,SN PRN Visits/Hours: 2 (disease exacerbation/labs)

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range

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Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Weight: < 209 or > 219
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:

• Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

• Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.

• Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.

• Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

• Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

GENITOURINARY:

• Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.

HOSPITALIZATION RISK FACTORS:

• Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: R SCAPULAR AND HUMORAL FRACTURE FROM FALL, HTN, CIRRHOSIS OF THE LIVER

• Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc

• Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.

• Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.

• Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.

• Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN:

• SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.

• SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications,

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vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.

• SN to instruct patient/caregiver on Heart Healthy diet.

60 Day Recert Summary

Physician: GAVIN MILLS MD
Date: 02/23/2026
Vital Sign Range:
Temperature: 97.5 - 98.8
Pulse: 67 - 93
Respiration: 16 - 18
Blood Pressure
Systolic: 110 - 120
Diastolic: 64 - 80
O2 Saturation: 95 - 99
Labs: none pending
Care Summary: FOCUS OF CARE: status post mechanical fall with right shoulder fx, weakness
Recertification visit was performed today for this homebound male who continues to receive skilled nursing services through Be Healthy Home Health due to complex medical history and chronic conditions. During the past 60-day certification period, the patient experienced two falls without serious injuries and no re-injuries to the right upper, neither fall requiring hospitalization or major interventions . They should continue to work on getting hearing aids as old ones were broken with follow ups in March with your doctors at the VA , patient states that he has had some weight loss and is working on his diet and increasing intake but states needed to lose some weight either way and it feels good about his progression status . Patient continues to slowly increase range of motion exercise exercises with right shoulder remains non-weight-bearing to the right shoulder at this time of the follow up ortho next week . Medication reconciliation performed with the following changes: no change changes to medication, no issues side effects or other complications
Hospitalizations, nurse visits, or doctors visits : patient is maintained ortho visits is working on follow up with your doctor for new hearing aids, waiting for urology referral and has a PCP follow up next week, patient has been compliant with visits and plan of care and doctors visit, .The patient remained free hospitalization during this certification period.
Assessment: patient was greeted at the door ambulating with use of hemi Walker, patient alert and oriented with no signs of cognitive deficits patient is very hard hearing and is using an auditory supplemental device until hearing aids can be corrected, patient and shortness of breath at rest and patient denies any chest pain patient has no edema or cardiovascular complications at this time, patient is consonant a bowel and bladder, no UTI symptoms however patient reports he has an ongoing issue with flow complications and has a pending urology consult in the next few weeks, patient stated that he had some weight loss but is working on diet and has stabilized and feels very comfortable and happy with where he is at with his weight, patient is up in ambulatory with the use of a hemi walker and use of a wheelchair for longer excursions, however patient remains no weight-bearing to the right upper arm and remains in a sling or supported when not in use or ambulating, patient has no skin breakdown or wounds, old scrapes from previous fall have healed completely very faint bruising to the right upper arm, patient is a mid-to mod ADLs at this time and remains a excellent candidate for ongoing skilled nursing education monitoring and assistance and progression with physical therapy as prescribed.
Skilled nursing interventions included cardiopulmonary assessments, medication reconciliation and education, monitoring for fluid status and electrolyte imbalance, and coordination of care with the physician and family. Education was reinforced fall prevention and reporting any falls and maintaining precaution safety with the right upper arm as well as injuries to any other areas, medication safety reporting assign symptoms of complications, and reinforce symptoms of UTI issues under report any

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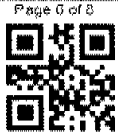
complications as soon as possible, pain management and overall plan of care patient verbalized compliance ongoing education reinforcement required. The patient remains appropriate for continued skilled nursing services in the next certification period, including Ongoing coordination of care with the physician and family. Patient was less safe and stable in-home
Pt is homebound due to having Multiple medical problems and functional impairment, the patient requires use of assistive devices and the assistance of another person in order to leave home. It requires considerable effort that is very taxing on the patient to leave home AEB multiple falls with injury and hospitalizations , patient with recent surgery for right total shoulder arthroplasty in September with a subsequent mechanical fall to the same resulting in a humeral fracture and a scapula fracture with the patient now being limited weight bearing to the right upper arm with the use of a assistive device and use of moderate assistance for ADLs combined with patients history of cirrhosis and cardiac comorbidities requiring taxing effort to leave home

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify in my estimation that the patient will need skilled care from home health for the next 60 days.
I attest to the following:
1. JOHN PERKINS is under my care.
2. I have received the following patient records from Be Healthy at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
3. Each of these patient records have been incorporated into my patient record.
4. These patients records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

Risk of Hospitalization

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications;cardiac comorbidity, RUE ROM, falls
Interventions: Referrals - SN,PT; Patient/Family Education; Front-loading Visits; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: More than 2 Secondary Diagnoses (M1022 & M1024); Inadequate Support Network (M 1100); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)



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Psychosocial Assessment

Depressed:Long Term,Treatment:oral Med;;
Primary Language -English;
Describe above abnormal findings:long-term oral therapy without acute complications at this time;
Sleep Rest:Adequate;
Other:patient is calm cooperative and engaged, patient denies HI and SI or active depression.

Advance Directive

POA; Copies Requested
Comments -: full code sister is POA
Patient's Advance Directive - Appointment of Health care agent;
Have an Advanced Care Plan and it is documented in the medical record: Yes - Medical Treatment Preferences;
Have a Surrogate Decision Maker and information documented in the medical record: Yes;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Medical restrictions;
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments: Pt is homebound due to having Multiple medical problems and functional impairment, the patient requires use of assistive
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22. Rehabilitation Potential/Discharge Plans/Goals :

Recertification of Patient Eligibility

I estimate the duration of continued Home Health services for this patient to be
60 days

Rehab Potential

Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

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Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

GENITOURINARY: - LTG
- LTG-Patient will tolerate collection of urine without pain and discomfort by end of episode.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG
- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
02/23/2026
Signed By: Rogers, Jesse - RN on
02/23/2026 11:04 AM, Cosigned by
Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
GAVIN MILLS MD
3551 ROGER BROOKE DR
JBSA Ft Sam Houston, TX 78234,NPI: 1477085934, License#: U4871,Phone: (210) 916-7144, Fax: (210) 916-1359

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:

03 Mar 2026

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.